

FAX TRANSMISSION

From: Fair Oaks Family Practice
To: Johns Hopkins Neurology Clinic
Fax: (410) 955-0000
Date: 03/14/2026 09:12 AM
Re: Referral – dizziness / possible TIA?
Pages: 12 (including cover)

Referral Reason: persistent dizziness, r/o central cause vs vestibular issue, pt still symptomatic
Requested: neurology eval (balance?)

Notes: pt seen multiple times, not improving fully. MRI done? (see attached) unclear findings per radiology. Family concerned about stroke.

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PATIENT DEMOGRAPHICS

Name: Carter, Melissa J
DOB: 08/22/1978
Age: 47
Sex: F
MRN: 5567382
Address: 14209 Pine Bluff Rd, Centreville VA 20120
Phone: (703) 555-1182

Insurance: CareFirst BCBS
Policy #: XJH8839201
Group #: 2201

Emergency Contact: Daniel Carter (spouse) 703-555-1183

Primary Care: Fair Oaks FP
Referring Provider: Daniel R. Kline, MD

Allergies: NKDA (pt reports “maybe penicillin rash as kid?” unclear)

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REFERRING PROVIDER NOTE

Date: 03/13/2026 16:02

Chief Complaint: dizziness

HPI:

47 y/o female with ~10 days of persistent dizziness described as “room spinning” initially, now more off-balance sensation. Began suddenly while getting out of bed approx 3/3. Associated with nausea, vomiting first 24–48 hrs (resolved). No syncope. No chest pain. No clear focal weakness though pt reports “legs felt weird” on day 1.

Seen in ED 3/4 (see attached), CT negative. Diagnosed with vertigo, given meclizine. Partial improvement but continues to feel unsteady especially with head movement. Reports difficulty walking in straight line earlier in course, improved but not baseline.

Denies hearing loss currently, though note from urgent care mentions “fullness L ear.” Denies tinnitus now.

Concern for possible posterior circulation event given prolonged symptoms? MRI reportedly done but report unclear (see attached—wording “no acute infarct” but limited?).

Exam today:

Vitals stable.

Neuro: CN II-XII grossly intact. No facial droop. Strength 5/5. Finger-to-nose ok. Gait slightly wide-based? Romberg equivocal. Dix-Hallpike attempted but patient very symptomatic, unclear result. No clear nystagmus observed today.

Assessment: persistent dizziness, unclear etiology

Plan: refer neurology for further evaluation, r/o central cause vs peripheral vestibular process

Meds: continue meclizine PRN

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PRIOR NOTE – EMERGENCY DEPARTMENT

Facility: Inova Fair Oaks Hospital

Date: 03/04/2026 11:47

Chief Complaint: dizziness, vomiting

HPI:

Pt is a 46 y/o female (DOB mismatch?) presenting with acute onset vertigo starting this morning at 6 AM. States “room spinning violently,” worse with head movement. Multiple episodes of emesis. Unable to stand without assistance. No prior similar episodes.

Denies headache initially but later reports mild occipital pressure. Denies vision loss. Denies numbness.

PMH: HTN, migraines, seasonal allergies
PSH: appendectomy
Meds: lisinopril, sumatriptan PRN

Exam:

BP 148/92

Neuro: alert, oriented. Horizontal nystagmus noted (direction not documented). Gait not assessed (pt unable). No focal deficits appreciated.

Imaging: CT head without contrast – no acute hemorrhage

ED Course:

Given IV fluids, ondansetron, meclizine. Symptoms improved slightly.

Disposition: discharged home

Diagnosis: vertigo (likely peripheral)

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PRIOR NOTE – URGENT CARE

Date: 03/07/2026 09:22

Chief Complaint: dizziness persists

HPI:

Pt returns for ongoing dizziness. States spinning sensation less intense but now feels “tilted” when walking. Reports left ear fullness and mild ringing (not present before per pt). No fever. No URI symptoms.

States ED told her it was “just vertigo” but she is worried because still not normal.

Exam:

Ears: TM normal bilaterally

Neuro: mild imbalance with tandem gait

Positional testing: “positive?” (note unclear)

Assessment: vertigo vs inner ear issue

Plan: continue meclizine, start prednisone taper (40 mg daily x5 days then taper)

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PRIOR NOTE – PRIMARY CARE (earlier visit?)

Date: 02/20/2026

Chief Complaint: headache

HPI:

Pt with history of migraines presents with 3-day headache, throbbing, photophobia. No dizziness at that time.

Assessment: migraine

Plan: sumatriptan PRN

(copy-forward section below)

PMH: HTN, migraine, GERD, anxiety

No history of stroke/TIA

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LAB RESULTS

Date: 03/04/2026

CBC:

WBC 9.8

Hgb 13.2

Plt 248

BMP:

Na 138

K 3.9

Cl 102

CO2 24

BUN 14

Cr 0.8

Glucose 112

LFTs: normal

TSH: 2.1

Troponin: negative

Urinalysis: trace ketones

Note: labs largely unremarkable

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IMAGING REPORT

MRI BRAIN WITHOUT CONTRAST

Date: 03/10/2026

Indication: persistent vertigo, rule out stroke

Findings:

No acute diffusion restriction identified. No intracranial hemorrhage. Ventricles normal in size. No mass lesion.

Comment: study somewhat limited by motion artifact. Posterior fossa structures grossly unremarkable though evaluation slightly degraded.

Impression: no definite acute intracranial abnormality identified

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CT HEAD WITHOUT CONTRAST

Date: 03/04/2026

Findings:

No acute hemorrhage, mass effect, or midline shift. Gray-white differentiation preserved.

Impression: negative CT head

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MEDICATION LIST (EHR EXPORT – may not be current)

lisinopril 10 mg daily
omeprazole 20 mg daily
sertraline 50 mg daily
sumatriptan 100 mg PRN migraine
meclizine 25 mg TID PRN dizziness
ondansetron 4 mg PRN nausea

prednisone taper (recent)
vitamin D 1000 IU daily
magnesium supplement (OTC)
melatonin PRN sleep

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DISCHARGE SUMMARY (ED)

Admit/Discharge Date: 03/04/2026 (same day)

Diagnosis: vertigo

Hospital Course:

Pt evaluated for acute dizziness. No evidence of stroke on CT. Symptoms improved with supportive care. Discharged with instructions to follow up PCP.

Condition at discharge: stable, ambulatory with assistance

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ADDITIONAL NOTE (triage call log)

Date: 03/11/2026

Pt called reporting still dizzy, “feels like on a boat.” No new weakness. Advised to continue meds and follow up.

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PROBLEM LIST (auto-generated)

Hypertension
Migraine headaches
GERD
Anxiety disorder
Dizziness (active)
Vertigo (active)

End of fax transmission

